

Fellow Logbook User Guide

How to log, review, and export your operative cases from your phone — for fellows.

AUDIENCE

Fellows on the roster

WHERE IT RUNS

LINE app, phone or tablet

SCREENS SHOWN ON

iPhone 17 · 402 × 874 pt

What this app is

An operative case logbook for your fellowship year, built for the ward and the OR rather than a desk. You open it from the Fellow Logbook LINE Official Account, log a case in a single scrolling form, and export the whole record as a PDF whenever you need it — for your programme, your portfolio, or a board submission.

Your cases are yours. The database restricts every row to the fellow who created it, so no other fellow can read your logbook, and you cannot read theirs. Staff at your institution see a read-only, HN-masked view of the cases logged there — described in the staff guide, not this one.

Three tabs, one screen

New Entry	The 14-question form for one operative case. Fourteen cards, top to bottom, most of them a single tap.
Case Log	Everything you have logged, newest first, with search, filters, and a count. Tap a case to open it, edit it, or delete it.
PDF	Export a month range as a formatted PDF — a summary page plus one block per case — built on your phone and handed to the iOS share sheet.

1	Getting in	Opening the app, and verifying your email the first time
2	When a screen stops you	The three refusals, and what each one is telling you
3	Logging a case	Questions 1–5: date, place, staff, HN, diagnosis
4	Classifying the fracture	Question 6 on the body map, question 7 for everything else
5	Finishing the entry	Questions 8–12, then images and your memo
6	Saving	What Save checks, what it does, and what Reset clears
7	Your Case Log	Searching, filtering, opening, editing, deleting
8	Exporting a PDF	Choosing a range, and what the document contains
9	Privacy, and quick answers	What leaves your phone, and a troubleshooting table

Getting in

The logbook lives inside LINE. There is no separate app to install, no username, and no password — your LINE identity plus a one-time email code is the whole of it.

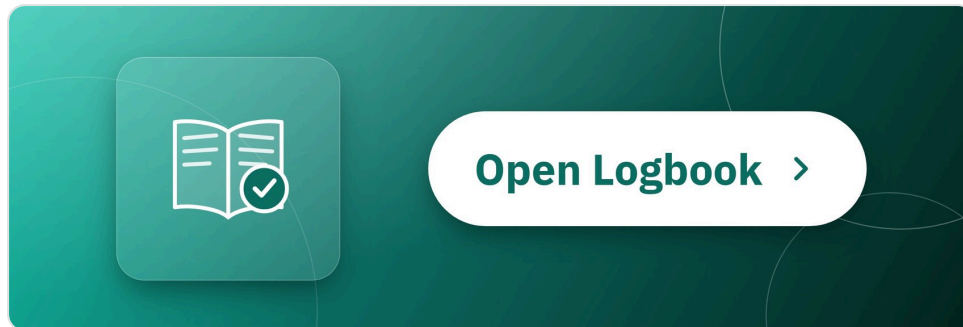


Figure 1.1 The rich menu at the bottom of the Official Account chat. It shows already expanded, and tapping anywhere on it opens the logbook.

Opening the app

- 1 Add the **Fellow Logbook** LINE Official Account, if you have not already. Your programme administrator will send you the link.
- 2 Open the chat. The rich menu sits at the bottom.
- 3 Tap **Open Logbook**. The app loads inside LINE's own browser.

WORTH DOING ONCE

Pin the Official Account to the top of your LINE chat list. The rich menu is the only entry point you need, and pinning it makes the logbook roughly as reachable as a home-screen icon.

PHONE OR TABLET, EITHER WAY

The same login works on your iPhone and your iPad, at the same time. Only the desktop LINE client is refused.

First time: verify your email

Only fellows on the programme roster can use the logbook, and the roster is keyed by email address. The first time you open the app, you prove you hold that address. This happens **once** — after it, your LINE identity is linked to your fellow record and you go straight into the app on every later open.

Verify Your Email

Enter the email address registered with your fellowship program. We'll send a one-time code to confirm it's you.

your personal email

Send Code

Figure 1.2 Enter the email registered with your programme — not necessarily your hospital address. Tap **Send Code**.

Enter Verification Code

We sent a 6-digit code to
thanawat.s@example.ac.th.

4

1

2

Figure 1.3 Type the six digits from the email. The boxes advance on their own, you can paste the whole code, and it submits itself on the sixth digit.

When a screen stops you

Three screens refuse to load the logbook. None of them is a fault in the app; each is telling you something specific, and each has a different fix.

FIRST, THE ONE THAT IS NOT A REFUSAL

If the verification code simply never arrives, check the junk folder. If it is genuinely missing, close and reopen the app to request a fresh one — codes expire, and an expired code reports itself as incorrect rather than as expired.

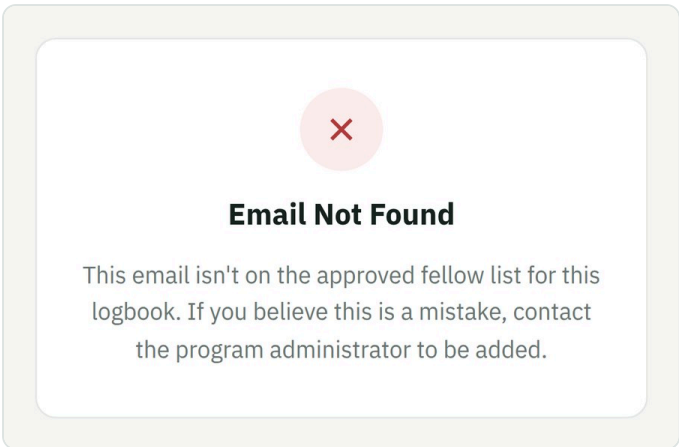


Figure 2.1 Email Not Found — the address is not on the roster.

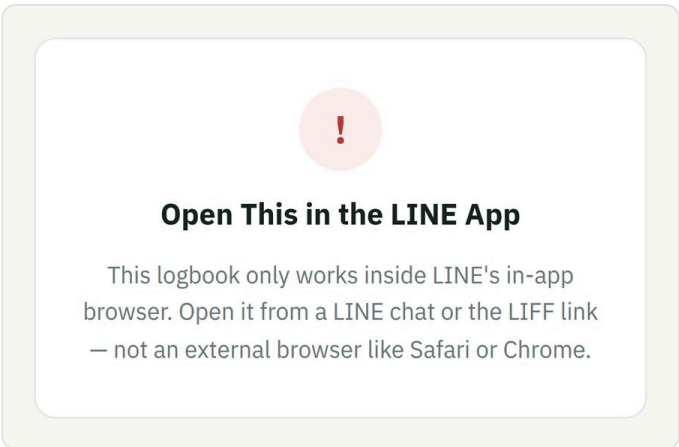


Figure 2.2 Open This in the LINE App — you are in Safari or Chrome.

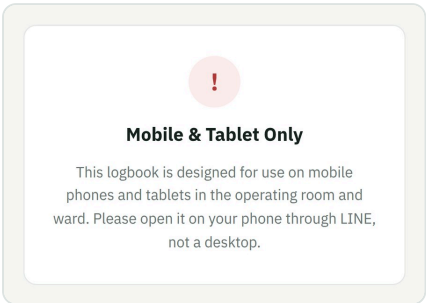


Figure 2.3 Mobile & Tablet Only — you are on the desktop LINE client.

Email Not Found	The address you typed is not on the approved fellow list, and no code was sent. Either it was a typo — reopen the app and retype it — or you are not on the roster yet, which only your programme administrator can fix.
Open This in the LINE App	You reached the URL from an external browser or a saved bookmark. The logbook runs only inside LINE's in-app browser. Go back to the Official Account chat and use the rich menu.
Mobile & Tablet Only	You are using the desktop LINE client on Windows or a Mac. The logbook is deliberately phone- and tablet-only, because that is where it gets used. Open it on your phone.
Something went wrong	A network or server problem rather than a rule. Tap Try again , which restarts the whole sign-in. If it keeps happening, tell your administrator — the failure is reported to them automatically, so they may already know.

YOU VERIFY ONCE, NOT ONCE PER DEVICE

After your first successful sign-in you will not be asked for your email again — not on a new phone, and not if LINE clears its cache. The app re-establishes your session from your LINE identity on its own. If you *are* asked a second time, you are almost certainly signed into a different LINE account than the one you verified with.

Logging a case • questions 1–5

New Entry is one scrolling form of fourteen numbered cards. A red asterisk marks a required field, and nothing saves until every one is answered — so work top to bottom and let Save tell you what you missed.

The screenshot shows the top of the 'New Entry' form. At the top, it displays the user's name 'นายแพทย์ ธนวัฒน์ ศรีวิไล' and the institution 'สสสมทศาสตร์'. Below this are three tabs: 'New Entry', 'Case Log (5)', and 'PDF'. The form consists of five numbered cards: 1. Date of Operation (with a date picker and 'Official hours'/'After hours' buttons), 2. Place (with 'Home institution' and 'Outside institution' buttons), 3. Staff (with a text field for 'Staff surgeon's name'), 4. HN (with a numeric keypad and example 'e.g. 1234567'), and 5. Diagnosis (with a text field).

The header

Your name and **Institution** are read from the roster, not typed by you — if either is wrong, tell your administrator. **Case Log (5)** counts what you have logged and ticks up the moment a case saves, which is the quickest confirmation that a save landed.

1 Date of Operation

Tap the date field and pick the operative date from the iOS wheel. Then choose the timing: **Official hours** **After hours**

Both parts are required. "After hours" is what makes an emergency workload visible in your exported summary, so it is worth getting right.

2 Place

Home institution **Outside institution**

Choose **Outside institution** for a case done anywhere other than your own hospital. Those cases still belong in your logbook and still appear in your export — the tag is only what lets them be counted separately.

Figure 3.1 The header carries your name, your institution, and the three tabs. It stays pinned as you scroll.

3 Staff

The staff surgeon you operated with, as free text. Type it the same way each time — counts group on exact wording, so อ. สมชาย and อาจารย์ สมชาย are two different people as far as the summary is concerned.

4 HN

The hospital number, **digits only**. The field strips anything else as you type and brings up the numeric keypad, so you cannot accidentally save an HN with a dash or a space in it.

WHY HN IS ASKED FOR AT ALL

It is how you find the case again months later when you need the films. It stays in your own rows: **never** in the exported PDF, only its last four digits to staff, and stripped entirely from any error report.

5 Diagnosis

Free text, and **multi-line on purpose**: press return and the next line starts as a new bullet, so a diagnosis written across several lines shows as a bulleted list wherever it appears later, the PDF included.

Write it as you would in an operative note — side, open or closed, and the bone or segment. Both the Case Log search and the export's region counts read this field, so specific wording pays off twice.

This screenshot shows the lower portion of the 'New Entry' form. It includes question 3 (Staff) with a text field, question 4 (HN) with a numeric keypad showing 'e.g. 1234567', question 5 (Diagnosis) with a text field containing 'e.g. Closed fracture right distal femur', and question 6 (AO/OTA classification) with a diagram and a dropdown menu. The diagram shows a bone with a fracture line and a circular region labeled '9' and '15'.

Figure 3.2 Staff, HN, and Diagnosis. The keypad for HN is numeric because the field accepts nothing else.

Classifying the fracture • questions 6 and 7

Questions 6 and 7 are a pair. Both cards are tagged **Answer #6 or #7**: the case saves as long as one of them has something in it. Answer both when both apply — that is the common case, not the exception.

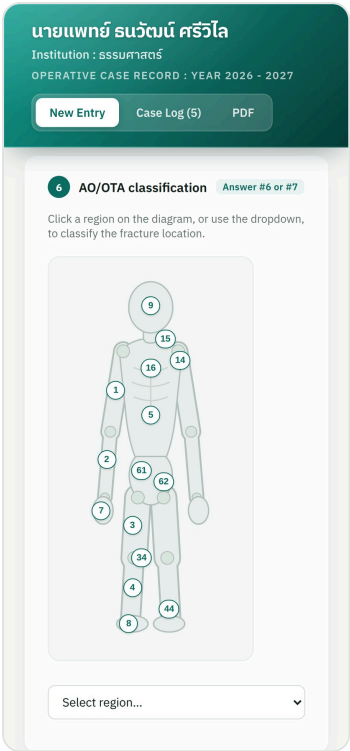


Figure 4.1 Sixteen anatomic regions, each marker labelled with its AO number.

6 AO/OTA classification

Rather than typing a code from memory, you narrow it down by tapping.

- 1 Tap the region on the body diagram — or use the **Select region...** dropdown underneath, which lists all sixteen by name. The two do the same thing; the dropdown is easier when markers sit close together, as they do around the pelvis and the ankle.
- 2 Answer whichever narrowing questions that region has. They differ by region: the femur asks only for a **Segment**; the forearm and the leg ask for a **Bone** first, then its segment; the hand and the foot ask for a **Specific bone** instead.
- 3 Pick the **Fracture type** — A, B, or C. Press and hold a type to see its description: simple / extra-articular, wedge / partial articular, complex / complete articular.
- 4 Add a **Group** (1, 2, or 3) if you know it. This one is optional — the code is valid without it.

The code assembles itself as you go and appears as a badge next to the question title, so you can see at a glance whether you built the code you meant to.

TWO ESCAPE HATCHES

The code box at the foot of the panel is **editable** — if the picker cannot express the code you want, type over it. And **Clear** resets the whole question, which is what you want after tapping the wrong region rather than undoing each pill in turn.

The sixteen regions, and what each one asks next

REGION	CODE	THEN ASKS FOR	REGION	CODE	THEN ASKS FOR
Craniomaxillofacial	9	type only	Pelvic ring	61	type only
Clavicle	15	type only	Acetabulum	62	type only
Scapula	14	type only	Femur	3	segment
Thorax	16	type only	Patella	34	type only
Spine	5	type only	Tibia / Fibula	4 / 4F	bone, then segment
Humerus	1	segment	Malleolus (ankle)	44	type only
Forearm (radius / ulna)	2R / 2U	bone, then segment	Foot	8	specific bone
Hand	7	specific bone	Fracture type A / B / C is asked for every region; group is always optional.		

Building the code, and question 7

A worked example: a diaphyseal femoral fracture, simple, group 2 — tapped out in four gestures.

Figure 4.2 Femur tapped. The marker fills in, the dropdown follows the diagram, and the badge beside the question title already reads **32-A2**.

Figure 4.3 The narrowing panel: segment, then fracture type, then the optional group. The computed code sits in an editable box at the foot, with **Clear** beside it.

7 Other Classification

Every classification that is not AO/OTA: Schatzker, Weber, Gustilo–Anderson, Judet–Letournel, Young–Burgess, Garden, Neer. Free text, and multi-line the same way Diagnosis is — one classification per line, and each becomes its own bullet.

In the Case Log and in the PDF, questions 6 and 7 are shown together under a single **Classification** heading: the AO/OTA code first, then each of your other classifications beneath it.

THE ONE VALIDATION YOU CAN TRIP HERE

If both 6 and 7 are empty, Save is blocked and the banner lists *"AO classification or Other classification"*. Answering either one clears it. This is the only place in the form where two questions are satisfied by one answer.

Finishing the entry • questions 8–12

Two free-text fields and three sets of options. This is the fastest part of the form, and it is where the numbers on your summary page come from.

8 Approach • 9 Procedure(s)

Both free text, both multi-line. Use a line per approach and a line per procedure when a case had more than one — a staged fixation with an external fixator and later plating reads far better as two lines than as one long sentence, both in your log and in the export.

10 Type of Procedure

Primary surgery Revision surgery Staged surgery

One choice. This drives a pie chart on the export's summary page.

Figure 5.1 Approach and Procedure(s) are the same multi-line, auto-bulleted fields as Diagnosis.

11 Your Role

Primary surgeon Primary assist Secondary assist Observer (not scrub in)
Uncertain

The single most consequential field in the form. It is charted on the summary page, and it is the axis of the **Experiences on Pelvis & Acetabulum** table, which counts your pelvic and acetabular cases by the role you held in each.

Record the role you actually had. **Uncertain** exists for the genuinely ambiguous case — use it rather than rounding up.

12 Operative Time

Skin to skin, as a range: < 1 hr 1–2 hr 2–3 hr 3–4 hr > 4 hr

An estimate is expected. The ranges are wide precisely so you do not need the anaesthetic record in front of you.

Figure 5.2 Role, in a two-column grid, and operative time.

Images and your memo • questions 13 and 14

The last two cards are optional, and both are tagged as such. A case saves perfectly well with neither.

13 Image(s)

Pre- and post-op films, intra-op findings. Tap **+ Add images** and pick from your camera roll or take a photo. JPG, PNG, and HEIC are accepted — iPhone HEIC files are converted for you — up to **10 MB in total** across all images on the case. The running total under the button turns red if you go over, and Save is blocked until you remove something.

Everything happens on your phone before anything is sent: each image is converted if needed, downscaled to a 2048-pixel long edge, and re-encoded. That keeps the upload small and, as a side effect, strips the photo's embedded metadata — camera GPS and timestamp.

Saved images go to the programme's own private Google Drive, never a public link, and are fetched back through the app when you open the case.

14 Memo

Notes for yourself: what went well, what you would do differently, an implant size worth remembering. Multi-line, same as the other text fields.

It is shown in your Case Log but is **not** included in the PDF export — so it is a genuinely private field, and a good place for the reflection you would not put in a submission.

The screenshot shows a mobile app interface for a medical case record. At the top, there's a header with the patient's name in Thai, the institution name, and the case record period. Below this are three tabs: 'New Entry', 'Case Log (5)', and 'PDF'. The main content area has two optional cards. The first card, '13 Image(s)', is labeled 'OPTIONAL' and contains a description of the image types, a '+ Add Images' button, and a note about the 10 MB total limit. The second card, '14 Memo', is also labeled 'OPTIONAL' and contains a text input field for notes. At the bottom of the form are 'Reset' and 'Save Case' buttons.

Figure 5.3 The two optional cards, and the actions at the foot of the form.

CHECK THE FRAME BEFORE YOU ADD IT

Nothing crops, blurs, or redacts an image for you. A patient name or a ward sticker visible in the corner of a photographed film will be stored exactly as captured, and staff at your institution can open that image. Frame it out, or photograph the film rather than the screen surrounding it.

IMAGES CAN WAIT

You do not have to attach films at the time. Save the case now, and add the images later from **Edit case** — the 10 MB ceiling then applies only to what you are adding, not to what is already there.

Saving

Two buttons at the foot of the form, and neither asks for confirmation.

Figure 6.1 Tapping **Save Case** with fields outstanding lists every one of them in a banner at the top of the form. Nothing is saved, and nothing you typed is lost.

Figure 6.2 A complete save: the toast confirms it, the form clears itself for the next case, and the tab count goes from 5 to 6.

What Save does

- 1 Checks every required field. Anything missing is listed in a banner at the top of the form, and nothing is written.
- 2 Uploads your images first, and only then writes the case — so a case is never stored without the films you attached to it. If an upload fails, nothing is saved and the message says why.
- 3 Confirms with **Case saved to logbook**, clears the form for the next case, and increments the count on the **Case Log** tab.

THE COUNT IS YOUR RECEIPT

If the tab count went up, the case is stored. A failed save gives you a message you have to dismiss yourself, rather than a toast that disappears while you are looking away.

RESET CLEARS EVERYTHING, IMMEDIATELY

Reset empties the whole form — every field, the AO selection, and any images you picked. There is no confirmation step and no undo. It is there for starting over, not for clearing one field.

Your Case Log

Every case you have logged, newest first. The tab title carries the count, so the log doubles as your running total for the year.

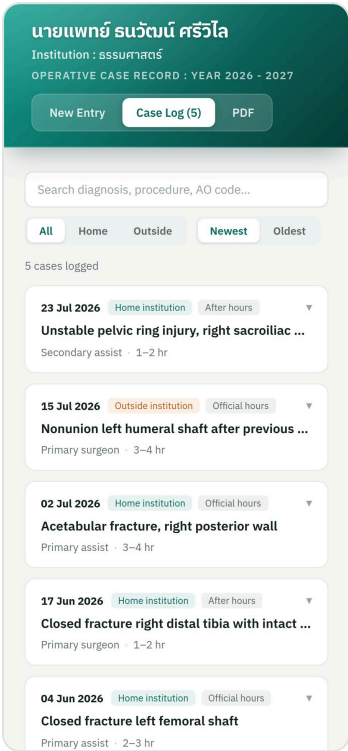


Figure 7.1 Search, two filter rows, the count, then the cards.

Reading a card

Each collapsed card shows the operative date, a **Home institution** or **Outside institution** tag, the timing, the diagnosis truncated to one line, and your role with the operative time. Enough to find a case without opening it.

Finding a case

- **Search** matches diagnosis, other classification, approach, procedure, AO code, and region name. Whole words in any order, and an AO code such as **43-B1** works as a search term. It does not search Staff, HN, or Memo.
- **All / Home / Outside** filters by where the case was done.
- **Newest / Oldest** flips the order. While a search is running, best matches lead instead, and this toggle takes over again once you clear the box.

The count line above the cards changes to *"3 of 5 cases"* whenever a search or filter is narrowing the list — which is how you tell a filtered list from a short one.

LOADED ONCE PER OPEN

Cases are fetched when the app starts, not continuously. If you logged a case on your iPad and want to see it on your phone, reopen the app on the phone.

Which control to reach for

"The distal tibia case from June"	Search tibia , then read the dates. Faster than scrolling, and the search box is the only control that looks inside the case text.
"How many of my cases were 43-B1?"	Search the AO code itself. The count line above the cards gives you the answer without opening anything.
"Everything I did away from my own hospital"	Clear the search and tap Outside . The count line reports the total against your whole logbook — <i>2 of 5 cases</i> .
"My year, in order, from the start"	Clear the search and tap Oldest . For anything longer than a quick look, export the PDF instead — it is the same record, laid out to read.

Opening a case

Tap anywhere on a card to expand it. The card scrolls itself to just under the header, so what you opened is what you are looking at. Tap it again to collapse.

นายแพทย์ ธนวัฒน์ ศรีวิไล
 Institution : ธรรมศาสตร์
 OPERATIVE CASE RECORD : YEAR 2026 - 2027
 New Entry Case Log (5) PDF

17 Jun 2026 Home institution After hours ▲
Closed fracture right distal tibia with intact ...
 Primary surgeon · 1–2 hr

STAFF
 อ. วิชัย
HN
 6698210
PLACE
 Home institution
HOURS
 After hours
DIAGNOSIS
 - Closed fracture right distal tibia
 - with intact fibula
CLASSIFICATION
 - AO/OTA 43-B1
 - Gustilo-Anderson type I
APPROACH
 Anteromedial
POSITION
 Supine
TYPE OF PROCEDURE
 Primary surgery
PROCEDURE(S)
 Open reduction and internal fixation with locking plate
MEMO
 Staff scrubbed for articular reduction.
IMAGES 3 images

Figure 7.2 The expanded view adds everything the card leaves out. Note the **Classification** block: the AO/OTA code from question 6 and the Gustilo–Anderson grade from question 7, as one bulleted list.

นายแพทย์ ธนวัฒน์ ศรีวิไล
 Institution : ธรรมศาสตร์
 OPERATIVE CASE RECORD : YEAR 2026 - 2027
 New Entry Case Log (5) PDF

Closed fracture right distal tibia
 - with intact fibula
CLASSIFICATION
 - AO/OTA 43-B1
 - Gustilo-Anderson type I
APPROACH
 Anteromedial
POSITION
 Supine
TYPE OF PROCEDURE
 Primary surgery
PROCEDURE(S)
 Open reduction and internal fixation with locking plate
MEMO
 Staff scrubbed for articular reduction.
IMAGES 3 images

Pre-op AP
 Pre-op lateral
 Post-op AP

Edit case Delete case

04 Jun 2026 Home institution Official hours ▼
Closed fracture left femoral shaft
 Primary assist · 2–3 hr

Figure 7.3 Images load from the programme Drive when you expand the case — tap a thumbnail for full size. **Edit case** and **Delete case** sit at the foot.

Expanded, a case shows staff, HN, place, hours, the full diagnosis, the combined **Classification** block, approach, position, type of procedure, the procedures performed, and your memo. Fields you left empty are simply absent rather than shown as blanks, so a short detail panel means a sparse case, not a display problem.

ONLY ONE CASE AT A TIME

Opening a second case closes the first. This is deliberate on a phone — it keeps the list scrollable instead of turning it into a wall of detail.

Editing and deleting

Both actions sit at the foot of an expanded case. One is entirely safe; the other cannot be undone.

Editing a case

Edit case reopens the case in the entry form, fully prefilled. The AO diagram and pills come back preselected from the stored code, so re-saving a case you did not otherwise touch cannot change its classification.

You can tell you are editing rather than logging: the first tab is relabelled **Edit Case**, a banner sits above the form, and the buttons become **Cancel** and **Save Changes**.

- Already-saved images can be removed, and new ones added, in the same edit.
- The 10 MB ceiling applies only to what you are adding. Images already on the case do not count against it.
- Saving returns you to the log with that case open, so you can check the change landed.
- **Cancel** abandons the edit and leaves the stored case exactly as it was.

There is no limit on how often you edit a case, and no record of the edit in your own view — so correcting a role or fixing a typo months later is entirely routine.

Figure 7.4 Edit mode: the relabelled tab and the banner above the form.

Deleting a case

THERE IS NO UNDO

Delete case removes the case immediately, along with its images. Nothing asks you to confirm, and the case cannot be recovered — if you delete one by mistake, you log it again.

The card disappears the moment you tap. If the server refuses the delete, the case **reappears** in the list and a message tells you it failed — so a card that comes back is a failed delete, not a glitch, and the case is still there.

Exporting a PDF

The PDF tab turns a month range of your logbook into a formatted document, built entirely on your phone.

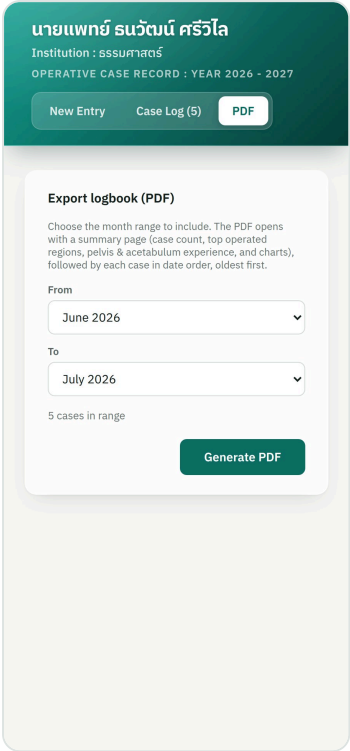


Figure 8.1 Two month pickers, a live case count, and one button.

Exporting

- 1 Open the **PDF** tab. **From** and **To** arrive already set to the full span of your logbook — for a whole-year export, change nothing.
- 2 Narrow the range if you want a quarter or a single month. Both pickers list only months you actually have cases in.
- 3 Check the line underneath: *"5 cases in range"*. If it reads 0, or if **From** is later than **To**, the button stays disabled and says so.
- 4 Tap **Generate PDF**. It reads *Building PDF...* for a few seconds — longer the first time, since the layout engine loads on demand.
- 5 The iOS share sheet opens with the finished file. **Save to Files**, mail it, or send it into a LINE chat yourself.

WHY THE SHARE SHEET

The file is generated on your phone and handed straight to iOS — it is never uploaded anywhere to be turned into a PDF. A LINE bot cannot attach a file to a chat, which is why nothing arrives in the chatbox on its own. If the share sheet does not appear, the app falls back to opening the PDF in your browser, and then to a plain download.

THE EXPORT IGNORES YOUR FILTERS

Whatever search or Home / Outside filter you left running in the Case Log has no effect here. The export is always the complete record for the months you chose.

If the export will not run

"No cases to export yet"	Your logbook is empty. Log a case first — there is nothing to summarise.
The button is greyed out	Either the range holds no cases, or From is later than To . The line above the button says which.
It says "Loading profile..."	Your name and institution have not arrived yet. It clears itself in a moment — the wait exists so a PDF cannot be built with a blank name on it.
"Could not export"	Try again. If it persists, tell your administrator; the failure is reported to them automatically.

What the document contains

A summary page, then your cases in order. Three things are deliberately left out.

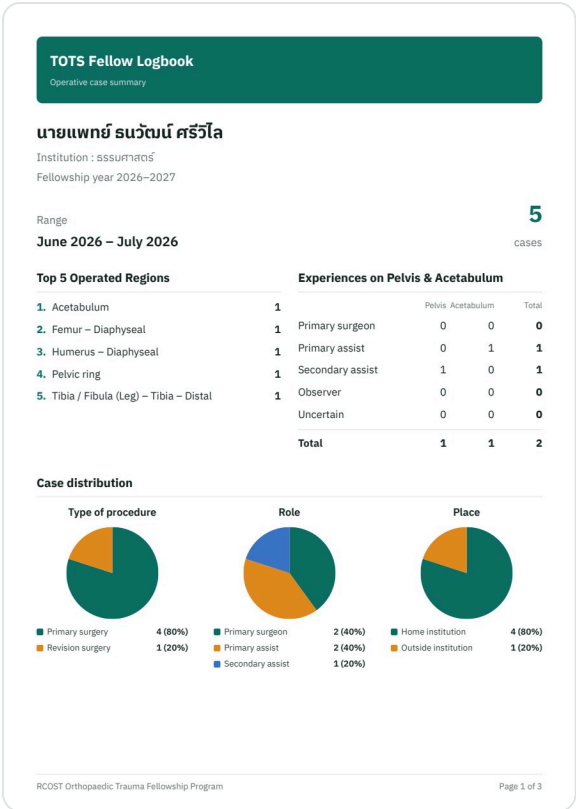


Figure 8.2 Page 1 of a real export: the case count, top operated regions, the pelvis and acetabulum table broken down by role, and the three distribution charts.

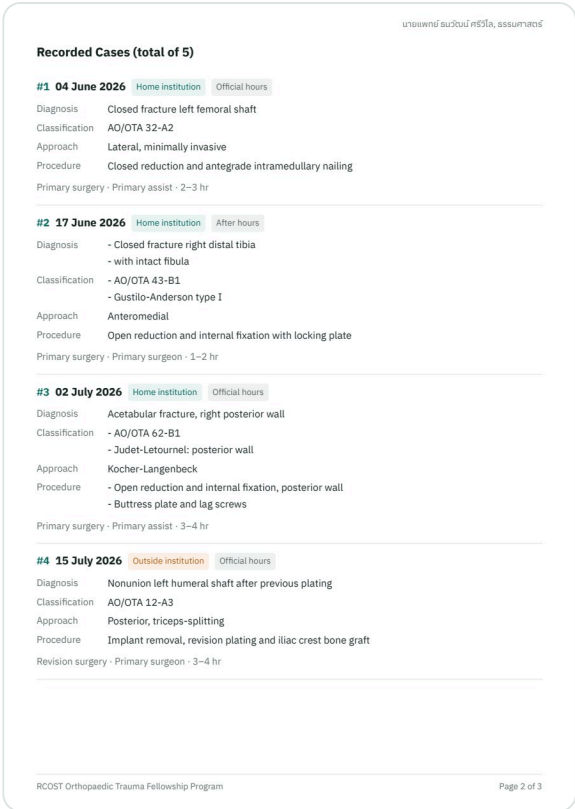


Figure 8.3 The pages that follow: one block per case, oldest first, with the date and the timing / place / AO-code chips.

Page 1 — summary Your name, institution, and fellowship year; the range and the case count; your **top operated regions**; the **Experiences on Pelvis & Acetabulum** table, broken down by the role you held in each case; and pie charts of **type of procedure**, **role**, and **place**.

Then, one block per case **Oldest first** — the reverse of the Case Log's default, because a logbook reads chronologically. Each block carries the date, the timing / place / AO-code chips, and questions 2 through 9.

Deliberately not included **HN**, your **memos**, and the **images**. The export is a record of your operative experience, not a patient record — which is what makes it shareable as it stands, without you having to check it first.

The file is named for you and the range, e.g. `TOTS-fellow-logbook_<your-name>_2026-06_to_2027-05.pdf`, so a folder of monthly exports sorts itself.

HABIT WORTH FORMING

Export at the end of each month and keep the file. It takes under a minute, it gives you a dated snapshot you can hand over without notice, and it means a portfolio deadline never depends on your logbook being complete that particular week.

Privacy, and quick answers

What the logbook holds, who can see it, and the questions worth answering before they come up.

Where your data lives

Your cases	In the programme's database, restricted at the database level to the fellow who created each row. Another fellow cannot read your cases even in principle.
Your images	In the programme's own private Google Drive, reached only through the app. They are never given a public link, and your phone never talks to Google directly.
HN	Stored on your own rows so you can find the case again. Masked to its last four digits anywhere it leaves them — the staff view, and your administrator's notifications. Never in the PDF, and never in an error report.
Staff at your institution	See a read-only list of cases logged by fellows at your institution, including yours, with HN masked and your name shown on each. They cannot edit or delete anything.
Your administrator	Receives a notification when a case is logged, edited, or deleted — with HN masked. Clinical text is included in full. This is programme oversight, not an audit trail.

YOUR PART OF IT

Two things are yours to get right, because nothing downstream can correct them: what you type into the free-text fields, and what is visible in the images you attach. Keep patient names and identifiers out of both — HN is the only identifier the logbook asks for.

Quick answers

Can I use it on my iPad?	Yes — open it from LINE on the iPad. Phones and tablets both work; only desktop is refused. You will not be asked to verify your email again on a second device.
I logged a case on my iPad and my phone does not show it.	Cases load when the app opens. Reopen it on the phone.
My name or institution in the header is wrong.	Both come from the programme roster and cannot be edited in the app. Ask your administrator to correct the roster.
Save says the images are too large.	10 MB is the ceiling across all images on one case. Remove one and save, then add the rest through Edit case — that allowance is separate.
I deleted a case by mistake.	There is no undo; the row and its images are gone. Log it again.
Nothing arrived in the LINE chat after I exported.	Expected. The PDF goes to the iOS share sheet, not into the chat. Use Save to Files , or pick a LINE chat from the share sheet yourself.
The app will not load at all.	Check you opened it from the rich menu inside LINE rather than a browser. If a "Something went wrong" screen appears, tap Try again ; if it persists, tell your administrator.

Screens in this guide were captured from the running app at iPhone 17 dimensions (402 × 874 pt). The fellow name, institution, staff names, HNs, and case details shown are fictional sample data. On a device with a different screen size, spacing and line wrapping will differ; the cards, their order, and their numbering will not.